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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°149 /MVE-BDBV/10/10/2026

Report date: October 10, 2026 Publication date: October 11, 2026

HIGHLIGHTS (last 24 hours)

- Over the past 24 hours, 64 new confirmed cases of Ebola Virus Disease (EVD), including 13 community deaths, have been reported. These cases originated from the provinces of Ituri (30 cases), North Kivi (25 cases), Haut-Uele (7), and Tshopo (2). In addition, 8 deaths were recorded in Ebola Treatment Centers (ETCs).
- A total of 445 new samples were analyzed; 64 of these samples tested positive for the Bundibugyo virus, representing a positivity rate of 14.3%.

ACCUMULATIVE CAS	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
8978	4 336	48,3 %	945	2 356	81,0%

Affected provinces and affected health zones

7 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele and Sud Ubangi

65 out of 167 health zones affected (38,9%)

Summary of key points from the last 24 hours

In the past 24 hours, **64 new confirmed cases** of Ebola virus disease (EVD) have been reported in four provinces: **Ituri (30 cases), the North Kivu (25 cases), Haut-Uélé (7 cases) and Tshopo (2 cases)**

In total, 21 confirmed deaths have been recorded, including 13 community deaths and 8 deaths in Ebola Treatment Centers.

Furthermore, **18 patients (11 in Ituri, 5 in Haut-Uélé and 2 in Tshopo)** were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has recorded **8,978 confirmed cases and 4,336 deaths,** representing a case fatality rate of 48.3%. In total, 65 out of 167 health zones (38.9%), spread across seven provinces, have been affected.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 10, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri		6639	3 074	46,3%	28/36 (77,8 %)
North Kivu		1883	1 075	57,1%	17/34 (50,0 %)
Haut-Uélé		379	159	42,0%	7/13 (53,8 %)
Boss		59	22	37,3%	7/23 (30,4 %)
South Kivu		3	1	33,3%	1/34 (2,9 %)
Bas-Uélé		13	4	30,8%	4/11 (36,4%)
Sud Ubangi		2	1	50,0%	1/16 (6,3%)
Total	**64**	**8 978**	**4 336**	**48,3%**	**65/167 (38,9%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 10, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**6639**	**3074**	**46,3%**	**30**	**6**	**3**	**9**
Give	12	1	8,3%				0
Understand	9	6	66,7%	1	0	0	0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	214	47	22,0%	1	0	0	0
have	2	2	100,0%				0
Bunia	1812	604	33,3%	8	3	0	3
Ladies	40	9	22,5%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	9	5	55,6%				0
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	178	107	60,1%	2	0	0	0
Liter	298	151	50,7%	1	1	0	1
Logo	14	7	50,0%				0
Fight	35	9	25,7%				0
distributed	4	1	25,0%				0
Washing	41	17	41,5%				0
Darkness	240	124	51,7%	5	1	0	1
Mangala	369	160	43,4%				0
Mongbwalu	712	341	47,9%	2	0	2	2
Nanny	324	181	55,9%	4	0	0	0
Nizi	809	290	35,8%	3	0	0	0
Legumes	133	39	29,3%				0
Jungle	18	5	27,8%				0

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Rwampara	1131	393	34,7%	3	1	1	2
Tchomia	76	25	32,9%				0
A valves	THAT	493	THAT				0
North Kivu	**1883**	**1075**	**57,1%**	**25**	**5**	**5**	**10**
Me	442	298	67,4%	5	3	2	5
Farm	23	7	30,4%				0
Butembo	297	185	62,3%	2	0	0	0
Ten	1	0	0,0%				0
Kalunguta	68	26	38,2%	1	1	1	2
Cut	623	379	60,8%	3	0	0	0
Source	3	2	66,7%				0
Kyondo	37	20	54,1%				0
Lubero	28	12	42,9%	5	1	0	1
Be anxious.	62	21	33,9%	6	0	0	0
Alimbong	5	3	60,0%				0
Manguredjipa	14	5	35,7 %	1			0
Masereka	36	12	33,3%		0	1	1
The museums	143	69	48,3%	1			0
Mutwanga	23	4	17,4%				0
Oicha	39	19	48,7%	1	0	1	1
Vuh	39	13	33,3%				0
Upper Uele	**379**	**159**	**42,0%**	**7**	**2**	**0**	**2**
Government Mangbetu	44	19	43,2%				0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	102	44	43,1%				0
Lining	98	47	48,0%				0
Beans	4	2	50,0%				0
Ordinary	127	45	35,4%	7	2		2
Boss	**59**	**22**	**37,3%**	2	0	0	0
Bafwasende	7	1	14,3%				0
Kabondo	6	1	16,7%				0
Lubunga	1	0	0,0%				0
Maquis - Kisangani	32	12	37,5 %	2	0	0	0
Mangobo	10	7	70,0%				0
Boss	2	0	0,0%				0
Wanie - Arugula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti -Murhesa	3	1	33,3%				0
Bas Uélé	**13**	**4**	**30,8%**	0	0	0	0
Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%				0
Walk	5	2	40,0%				0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Total	**8978**	**4336**	**48,3%**	**64**	**13**	**8**	**21**

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*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified as of October 10, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	87	5	92	15	5	72	0	20	3
Bas Uélé	108	4	112	26	4	82	0	30	0
Ituri	ND	ND	ND	ND	ND	ND	ND	ND	ND
North Kivu	1 332	69	1 401	187	69	1 145	0	256	138
South Bank ND 108		ND	ND	ND	ND	ND	ND	ND	ND
Boss		0	108	4	0	104	0	4	2
Total General	**1 635**	**78**	**1 713 232**		**78**	**1 403**	**0**	**310**	**143**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

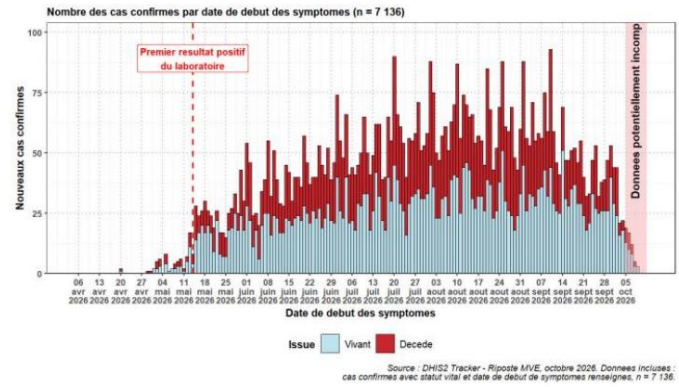
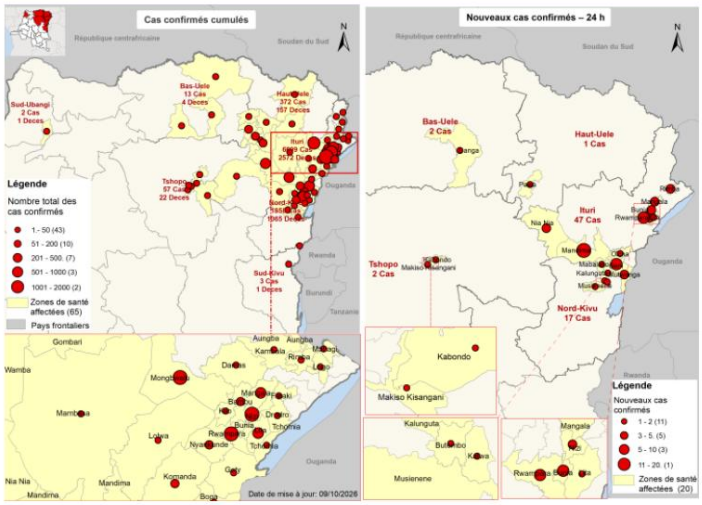


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1.COORDINATION

1.1. Main Actions

• Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

• As of October 10, 2026, 1,713 alerts were reported in four provinces (North Kivu, Haut-Uélé, Bas-Uélé, and Tshopo); Ituri and Sud-Ubangi did not provide updated data (ND). 1,713 alerts were verified (97.6%), of which 310 (18.5%) were validated as suspected cases. All 310 suspected cases were investigated (100.0%). Among them, 143 (46.1%) were transferred to treatment centers/infection control centers.

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• Of the 19,346 contacts being monitored in the provinces that submitted their data, 15,672 were seen in the last 24 hours, representing a follow-up rate of 81.0%, down from the previous day (83.1%) and below the required threshold of 95.0%. By province, this rate was 95.1% (1,843/1,938) in Haut-Uélé, 92.9% (223/240) in Bas-Uélé, 88.9% (585/658) in Tshopo, and 78.9% (13,021/16,510) in Nord-Kivu; Ituri and Sud-Ubangi did not submit updated data (ND). Only Haut-Uélé reached the 95% threshold.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

• Seven (7) alerts were notified to the PoE/PoC on October 10: six (6) in Ituri and one (1) in North Kivu. In Ituri, of the five (5) validated live alerts, only one was referred to the Elikya CTE (Mudzi Bala PoC); two were not referred to the Mambasa CTE due to lack of space, and two patients refused transfer; the lifeless body of a newborn intercepted at the DGDA checkpoint (Mahagi Health Zone) was swabed (result pending). In North Kivu, a lifeless body was intercepted at the Kasindi PoE and sent to the Lubiriha Health Center for swabing.

Table 4. Status of key PoE/PoC indicators by province, as of October 10, 2026

Indicators	Ituri	North Kivu	Boss	Global
Report completeness	93,4% (57/61) 95,2% (20/21)		100,0% (18/18)	95,0% (95/100)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	140 888	93 282	18 212	252 382
% of travelers screened	97,8%	98,4%	97,8%	98,0%
% of travelers who washed their hands	97,7%	98,4%	97,6%	97,9%
% of travelers made aware	99,5%	98,4%	100,0%	99,1%
Number of alerts notified	6	1	0	7
Number of validated live alerts (suspected cases)	5	0	0	5
Number of bodies recovered today	1	1	0	2
% of suspected cases transferred to CTE/CT	20,0%	THAT	THAT	20,0%
% of lifeless bodies swabed/secured	100,0%	100,0%	THAT	100,0%
% of suspected cases investigated within 4 h	80,0%	THAT	THAT	80,0%

2.2.2. Challenges

In Ituri, the Avakubi and Zunguluka PoCs remain non-functional due to insecurity, and the Makoko 2 PoC has not reported any cases. In North Kivu, the Butuhe PoC has remained non-functional since September 26, 2026. In Tshopo, 2.2% of travelers refused screening and 2.4% refused handwashing. In Bas-Uélé, only 19 out of 41 notification sites are functional (46.3%).

2.3. Laboratory

2.3.1. Main actions

• **Ituri: 30 new positive results** (24 living and 6 deaths) out of 198 samples received and analyzed **(positivity: 15.2%).**

• **North Kivu: 25 new positive results** (20 living and 5 dead) out of 220 samples received and analyzed **(positivity: 11.4%).**

• **Haut-Uélé: 7 new positive results** (5 living and 2 deceased) **out of 13 samples received and analyzed (positivity: 53.8%).**

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- **Tshopo: 2 new positive results (2 blood tests) out of 7 samples received and analyzed (positivity: 28.6%).**

- **Bas Uélé: 7 new** samples received **(all live),** all tested negative

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 25 out of 42 rings were opened (59.5%), 44 out of 60 households were decontaminated (73.3%), and 9 out of 9 health facilities were decontaminated (100%); no households, social economy enterprises, or schools received kits. Of 108 blood and sanitation alerts (including 8 postponements from the previous day), 70 blood and sanitation surveys were carried out (64.8%), and 82 bodies were swabbed, with 23 failures (21.3%) due to resistance in Bambu, Mangala, and Mandima.

- **North Kivu:** 26 out of 36 infection control rings were opened (72.2%), with a gap in Katwa, Mabalako, and Beni; 25 out of 27 health centers (92.6%) were decontaminated, and 17 out of 43 households (39.5%) were decontaminated. None of the decontaminated households received infection control kits. Seven infection prevention and control (IPC) assessments (scorecard) were conducted in seven health centers, with an average score of 37.8%, and no people with HIV were infected among the 17 confirmed cases (0/17). Out of 101 expected deaths (64 alerts and 37 postponements), 53 death assessments were carried out (52.5%), and 64 bodies were successfully swabbed, with 48 postponements to the following day. Handwashing kits were distributed in 52 schools in five health districts (AS) of the Beni health zone with support from CUB/UNICEF.

- **Haut-Uélé:** 1 out of 1 ring opened (100%) around the Pawa case, 7 out of 7 households decontaminated (100%), and 4 out of 4 ESS (100%), including 1 in Isiro and 3 in Pawa; 6 ESS were assessed using the scorecard method (scores ranging from 6.5% to 56%, none reaching 80%), 80 service providers were briefed (33 in Pawa, 26 in Isiro, 18 in Wamba, and 3 in Boma Mangbetu), and 3 Category 3 IPC kits were packaged for the Bolebole health center. Of 6 EDS alerts, 3 EDS were carried out (50.0%), and 3 bodies were swabed.

- **Tshopo:** 11 out of 11 COVID-19 treatment centers (DTCs) were monitored in 5 health zones, and 142 healthcare providers were briefed on standard precautions, triage, and screening. The Shoukrani Community Health Center (4 rooms and 1 latrine, around the 57th confirmed case), the Kawaki Clinic (3 rooms and 1 latrine), and the household of a confirmed case (10 rooms and 2 latrines) were decontaminated. The provincial cumulative total reached 45 out of 45 DTCs open (100%) and 34 out of 37 households decontaminated (91.9%).

2.4.2. Challenges

- **In Ituri,** the opening of the rings remains insufficient (59.5%), there are insufficient teams in Komanda, Mangala and Mandima, mobility is lacking in Komanda, ECUMER coverage, EDS inputs and means of transport (Kilo, Nia-Nia, Mandima, Damas, Mambasa and Fataki) are insufficient and 23 resistances to EDS have been recorded.

- **In North Kivu,** the gap in the opening of the rings in Katwa, Mabalako and Beni, the low decontamination of households (17/43; 39.5%), the lack of provision of kits to decontaminated households, an average score card of 37.8% (no ESS out of 7 above 80%) and the high risk exposure of 14 out of 77 staff assessed persist.

2.4.3. Holistic care

2.4.4. Main actions

- **In Ituri,** 80 new admissions were recorded against 59 discharges, including 11 recoveries, 40 non-cases and 8 deaths (3 of which were confirmed). 336 patients remained in isolation at the end of the day, representing a bed occupancy rate of 35.5% (360/1015), including 33 critically ill patients; 345 out of 350 patients (98.6%) received a hot meal and all children under LNPE/SLM were cared for (10/10 at the CTE, 6/6 at the nursery and 52/52 in the community).

- **In North Kivu,** 103 new admissions were recorded compared to 125 discharges, including 5 recoveries. In total, 501 patients remained in isolation at the end of the day, representing a bed occupancy rate of 94.9% in CTE/CTs, including 19 critical patients at the Beni CTE; 148 patients (31.2%) are being treated in ESSs that are not CTE/CTs, and 282 patients, 208 companions and 391 PPLs received hot meals.

- **In Haut-Uélé,** 7 new admissions (1 confirmed and 6 suspected) were recorded, compared to 13 discharges (5 recoveries and 8 negative cases), with no deaths. A total of 68 patients remained in hospital.

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isolation at the end of the day, i.e. an overall bed occupancy rate of 29.4% (45/153), including 2 severely or critically ill patients at the Pawa CTE; 55 patients, 67 accompanying persons and 70 PPL received hot meals.

- **In Tshopo,** 1 new admission (1 suspected case) was recorded, compared to 2 discharges, all recoveries, including one doctor. In total, 14 patients remained in isolation at the end of the day, representing an overall bed occupancy rate of 30.6% (11/36), including 1 patient in serious condition; since the beginning, 439 people have received the experimental treatment, including 40 confirmed cases, and 399 have benefited from post-exposure prophylaxis.

- **In Bas-Uélé,** 26 new suspected cases were admitted to transit centers (Buta 6, Bondo 4, Ganga 4, Poko 3, Titule 3, Likati 2, Viadana 2, Aketi 1 and Dakwa 1), with no discharges; in total, 26 patients are in isolation and no new confirmed cases were admitted.

2.4.5. Challenges

In Ituri, the low capacity of some CTE/CT (Mambasa, Lolwa, Mandima and Biakato), the care of 33 critical patients and the lack of food for 5 patients at the Mandima CT remain worrying.

In North Kivu, the saturation of suspected beds (158/165; 95.8%), the care of 148 patients (31.2%) outside of CTE/CT and the low completeness of ambulance data (57%) remain worrying.

In Haut-Uélé, the escape of 2 confirmed cases from the Pawa CTE, inconsistencies in care data and the absence of a hearse at the Boma Mangbetu CTE remain challenges to be solved.

In Tshopo, the demotivation of 12 out of 29 ambulance workers (41.4%) undermines the transfer of patients.
In Bas-Uélé, the lack of a standardized CTE, still under construction, and of an ambulance for the transfer of patients remains a major operational challenge.

2.5. Vaccination

2.5.1. Main actions

- **In Haut-Uélé,** the cumulative total reached 399 PPL vaccinated out of 858 pre-registered (46.5%), with 399 doses of Ervebo vaccine used out of 500 received; the retro-entry of vaccination data into the DHIS2 Tracker began after the training of data managers from the DPS, the PEV branch and the ZS.

- **In Ituri,** data collection for the study on the impact of the EVD epidemic on the use of sexual and reproductive health services continued in the health zones of Mongbwalu, Bunia, Rwampara, Tchomia and Nyarambe, as did qualitative and operational surveys and the Partners, Oracle and EBO-PEP clinical trials.

2.5.2. Challenges

- **In Ituri,** vaccination of PPL and the study of cross-immunity with the BDBV strain remain suspended pending the supply of Ervebo vaccine by the national PEV.

- **In Bas-Uélé,** the implementation of the UCCs is not effective and the micro-plan has not been validated, as well as the Vaccination funds are still not available.

2.6. Continuity of Care

2.6.1. Main actions

- **In Tshopo,** the evaluation of the Cliniques des Anges de Kisangani (CAKIS) showed an effective triage area, an operational incinerator and systematic use of the partogram, but only 6 agents vaccinated against Ebola out of 21 (28.6%); the recovery plan of the CS RTNC was developed, and the follow-up of PPL at the CTM and the CTE as well as that of recovered patients with comorbidities at the HGR of Makiso-Kisangani continued.

- **In North Kivu,** the encoding of assessments of the continuity of essential health services in the health establishments of the Butembo health zone continued.
- **In Bas-Uélé,** no data has been transmitted by the continuity of care pillar in the last 24 hours.

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2.7. Risk Communication and Community Engagement (RCCE)

2.7.1. Main actions

- **In Ituri,** 21,339 home visits reached 61,212 people; 663 educational talks brought together 10,383 people, 20 mass awareness campaigns reached 2,665 people, 7 community debates involved 217 people, and 11 advocacy sessions engaged 19 authorities and leaders; 28 public service announcements were broadcast on 36 radio stations, and 17 programs were organized. A total of 388 community alerts, including 91 deaths, were reported. 250 out of 282 feedback requests (88.7%) and 12 out of 18 incidents (66.7%) were resolved within 48 hours. 35 community health workers (CHWs) from the Nzere health district were briefed on the village approach and community mobilization cells. 44 teachers and 987 students from the Kibonge Institute (Komanda health zone) were sensitized, and 151 contacts were pre-listed around 5 confirmed cases. The completeness rate of the CREC reports is only 39.3% (11/28 health zones).

- **In Bas-Uélé,** 2 mass awareness sessions reached 250 people in the Limba AS (Viadana ZS) through a motorized caravan with testimonies from those who had recovered; a radio broadcast reached 9,650 people in Likati; 50 people (including 30 RECOs, 10 base chiefs and 10 female leaders) were briefed in Viadana and 110 people were sensitized in Dakwa; 4 people who had recovered were reintegrated into the community in Viadana).

- **In Tshopo,** 183 home visits reached 1,855 people, and 3 educational talks reached 173 people and 6 mass awareness sessions reaching at least 162,000 people, notably through a motorized caravan in the communes of Makiso, Mangobo, Tshopo, Kabondo and Kisangani; 30 community alerts were notified and all dealt with, the feedback received was resolved (1/1) and 250 learners and 15 teachers of the Kimbanguist ITM were sensitized in Lubunga.

- **In Haut-Uélé,** 13 educational talks brought together 312 people, 3 mass awareness sessions reached 1,357 students and 45 home visits reached 225 people in Wamba, and 45 RECOs from the Bolebole AS were briefed on the complete Ebola package, reaching 1,939 people; 5 community alerts, including 4 deaths, were notified in Wamba, all 5 feedback received were resolved (100%), and 61 family members of 4 suspected deaths were sensitized to contact tracing, decontamination, and EDS.

- **In North Kivu,** 10,939 home visits reached 31,720 people, 610 educational talks reached 4,478 people, 48 mass communications reached 4,832 people and 5 briefings reached 184 people; 396 community alerts were notified out of 650 expected, 22 feedbacks out of 30 (73.3%) and 3 incidents out of 6 (50.0%) were resolved within 48 hours, community dialogues involving local leaders were launched in Katwa and 51 school officials were briefed in Lubero.

2.8. Mental Health and Psychosocial Support (MHPSS)

2.8.1. Main actions

- **In North Kivu,** 16 out of the last 47 confirmed cases (34.0%), 68 confirmed cases currently being monitored out of 76 (89.5%), 36 new suspected cases out of 60 (60.0%), and 92 suspected cases currently being monitored out of 101 (91.1%) benefited from SMSPS interventions. The teams supported 226 caregivers and 73 people living with HIV/AIDS (PLHIV), monitored 21 out of 26 recoveries (80.8%) in the community and 13 out of 13 children in daycare, assisted 9 out of 13 separated and orphaned families, reintegrated 6 out of 8 recoveries, facilitated 1 family reunification, and announced 36 results, including 16 positive ones; 47 children are currently being monitored.

- **In Tshopo,** 2 out of 2 most recent confirmed cases (100%), 4 confirmed cases under follow-up out of 4 (100%), 2 new suspected cases out of 2 and 7 suspected cases under follow-up out of 7 (100%) have benefited from SMSPS interventions; 3 households have been accompanied, 4 PPLs supported, 2 recovered individuals followed up in the community and reintegrated, 160 people have benefited from psychoeducation and 7 results, including 2 positive ones, have been announced; the 3 notified resistances have been lifted and 5 out of 7 health zones that notified are covered.

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- **In Haut-Uélé,** 3 new confirmed cases, 23 confirmed cases under observation, 6 new suspected cases and 28 suspects under monitoring benefited from SMSPS interventions in the CTEs of Isiro, Pawa, Boma Mangbetu and Wamba; the teams supported 31 caregivers, 63 PPLs, 10 children separated in the community and 9 household members, supported 1 EDS and announced 19 results, including 3 positive ones

2.8.2. Challenges

- **In Ituri,** only 14 out of 28 affected health zones are covered (50.0%); the health zones of Mandima and Rimba concentrate 47% of notified cases not covered, which requires strengthening interventions around the cases, particularly in Nizi and Mangala.
- **In Bas-Uélé,** SMSPS coverage remains embryonic, with activities having only just begun in the Viadana ZS.
- **In Haut-Uélé,** the mobility of community health workers remains difficult in Wamba and Boma Mangbetu, nutritional kits are lacking for weaned infants in Wamba and Isiro, the community health worker lacks computer equipment, and data collection remains weak in Boma Mangbetu and Pawa
- **In Tshopo,** there is a need to train the APS of the ZS of Wanie-Rukula and Bafwasende and a shortage of communication units and motorcycles for the movement of the pillar.

2.9. Logistics

2.9.1. Main actions.

- Ensure the mobility of all field teams in all affected provinces.

2.10. Security

2.11. Main actions

- **In North Kivu,** teams were escorted to the burial of a confirmed case at the Bundji cemetery, the decontamination of a household in the Kasanga health district and the Arche dispensary (Ngongolyo health district), the swab of a deceased person at the Horizon morgue, and a situation assessment of the Kambuli health district (Katwa) with partner FHI 360; a demonstration session on EDS practices was also organized in Kambuli with the risk mitigation team

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

- **In Ituri, the** training of trainers on PSEA-H/AAP, held from October 6 to 10, 2026 in Bunia, was closed with a theoretical and practical evaluation and the awarding of certificates; 18 people (14 women, 3 men and 1 boy) were sensitized on PSEA-H and the reporting channels (151 and 495555) in the AS of Bakoko, and the posting of messages and the distribution of leaflets continued in Bunia, Aru, Mongbwalu, Mambasa and Fataki.
- **In North Kivu,** awareness sessions on the PSEAH (Prevention of Sexual Abuse in the Humane Environment) and reporting via the 151/495555 hotline reached 35 people in the Mabolio health district (Beni health zone), 13 students from the Visonge Institute (Butembo health zone), 54 people from the Mabanga neighborhood (Mutwanga health zone), and 66 churchgoers from the Kamina neighborhood (Manguredjipa health zone). The Mutwanga Immunization Prevention (IPC) staff also received refresher training on combating sexual misconduct. In Haut-Uélé, four members of the IMA (Integrated Medical Assistance) team were briefed on the PSEAH and signed the code of conduct in Wamba. Four awareness sessions were also held in Boma, Pawa, and Isiro for students from the Malamu Institute, drivers involved in the response, and community health workers.

2.12.2. Challenges

At the national level, only 3 out of 7 DPS (42.9%) reported their PSEA activities (Ituri, North Kivu and Haut-Uélé); the proportion of ZS aware of the reporting mechanism remains low in Ituri (36.1%; 13/36), North Kivu (17.6%; 6/34) and Haut-Uélé (46.2%; 6/13), visibility supports are insufficient and some partners are not yet aligned.

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MESSAGES CLES

ÿ In the provinces of Ituri, North Kivu and Haut-Uélé, there is active community transmission while sporadic notification is observed in the province of Tshopo.

2. In the last 24 hours.

ÿ Over the past 24 hours, all suspected cases (100.0%) have been investigated; performance is consistent with data reported by the provinces. However, contact tracing remains inconsistent (81.0% vs. 83.1%) and warrants close attention to prevent uncontrolled chains of transmission.

Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care structures with adequate reception capacities.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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